

LXEO deck

PITCH — LXEO — VERDICT

PROPOSED — PENDING ADJUDICATION — no market order; a ladder starter of up to 0.5% of book via resting limits at \$4.25 and \$3.75, with zero sensors currently armed and the entry levels sitting inside the issuer's own live selling window.

What we believe

Lexeo Therapeutics is a clinical-stage gene therapy company with two cardiac programs and no revenue. The stock is at **\$5.06** (the two benches worked off \$5.13 intraday; the evidence pack's later tape print is \$5.06 — the difference does not change any conclusion, but every number below is shown at both where it matters). That is 54% below its 52-week high of \$10.99 and 27% above its 52-week low of \$3.99. So this is a name that fell hard and has partially bounced — not a fresh break.

What we actually believe, stated plainly:

- 1. The company is worth roughly \$220–226 million net of cash.** Fully diluted for the 7.35 million pre-funded warrants the cover page of the 10-Q leaves out, the share base is about 89.4 million, not the 82.0 million on the cover and not the 43.6 million our pack's stale XBRL field reported. At \$5.13 that is a \$459M market value less roughly \$233M of net cash, so about **\$226M enterprise value**; at \$5.06 it is about **\$219M**. The original brief that started this court said \$180M. That was wrong by roughly 25%, and it was wrong in the direction that made the idea look cheap.
- 2. At that price the market is paying about a 15–21% chance that the lead program works.** If you assume the Friedreich's ataxia cardiomyopathy program (LX2006) were approved and fully funded — 3,000 addressable patients in the US and EU, 50% penetration over ten years, \$2.5M one-time price, 70% operating margin, 12% discount rate — the present value is about \$19.25 per share. Today's enterprise value divided by that prize implies roughly a 15% probability of success. Adjust for the fact that approval realistically arrives in 2029, not tomorrow, and the implied probability rises to about 21%. Adjust in the other direction for the fact that the second program (LX2020, for PKP2 arrhythmogenic cardiomyopathy) addresses a much larger patient pool and is currently being valued at zero in that math, and the implied probability falls again. **The honest answer is a band of roughly 10–25%.** The industry base rate for a Phase 2 asset reaching approval is about 15%. The band contains the base rate. We cannot tell from price alone whether this is cheap or fair.
- 3. The one thing that is not ambiguous is supply.** The company sold stock in April at \$5.65 and again in July at \$4.70 — a lower price, five weeks before this court convened. It has used \$39.6M of a \$75.0M at-the-market program, leaving about **\$35.4M of live selling capacity**,

which at \$5.13 is another 6.9 million shares, or about 7.7% more stock. On top of that sit 7.35 million pre-funded warrants that are economically free-trading shares which never appear on the share-count line most people read.

4. **Therefore: no market buy.** The defensible position is a small, patient bid below the level management has demonstrated it will sell at, sized so that being wrong costs us little, held as an option on a specific data readout rather than as a valuation trade.

What the market believes

The market believes roughly what we believe, which is the core problem with this idea.

- Four specialist healthcare funds filed ownership disclosures between August 7 and August 14: Balyasny at 9.99%, Affinity at 8.4%, Paradigm at 5.3%, OrbiMed at 5.0%. These are exactly the funds that read gene therapy 10-Qs. Balyasny took pre-funded warrants in the October 2025 offering at \$7.9999 and is roughly 36% underwater.
- The July at-the-market sale is disclosed in a 10-Q filed August 12 that all of those holders have read. Our red bench initially tagged it a *novel* finding; our blue bench correctly re-tagged it **consensus** — it was novel to *us*, not to the tape.
- The only external price anchor either bench could produce is a J.P. Morgan target cut from \$12 to \$9. That is roughly 75% above the current price. So the sell side is more optimistic than we are, not less.
- The stock's 27% bounce off the low tells us someone is already buying this drawdown.

In short: the market is applying a normal failure probability to an early-stage asset, in a name that specialists already own, and the published research is more bullish than our own math. There is no obvious group of forced or uninformed sellers here.

Why we might have edge

Four candidate sources, ranked by how much we actually trust them:

1. **Cap-structure arithmetic that most screens get wrong (moderate confidence).** The 10-Q cover page says 82,046,255 shares. The company's own Q2 weighted-average share count — both basic and diluted — is 85,626,751, which is *higher* than the cover count. That is only arithmetically possible if the penny warrants are inside the earnings-per-share denominator but outside the cover count. Any screen that computes market value from the cover page understates this company by about 9%. That is a repeatable error we can detect systematically, and it is the basis of a proposed new detector (see "What remains unverified").
2. **The timing of the issuance pattern (moderate confidence).** Selling at \$5.65 and then at \$4.70 into a drawdown, while a press release the same day markets a "runway into 2028," tells you something about how management views its own cost of capital. That is not a secret, but it is the kind of thing that shows up in a subsequent-events footnote rather than a headline.

3. **A free external read-through on the second program (moderate confidence, and genuinely useful).** Rocket Pharmaceuticals' RP-A601 and Tenaya's TN-401 are competing PKP2 gene therapies. Rocket has reported raised PKP2 protein on cardiac biopsy in all three Phase 1 patients — an independent replication of the biological mechanism LX2020 depends on. That means competitor data releases are a tradeable information channel for LXEO that does not require LXEO to say anything. It cuts both ways: it validates the mechanism and it contests the market.
4. **Patience as the edge (low confidence, but honest).** We have no position, no deadline, and no client pressure. If the correct answer is “wait for a specific piece of data at a specific price,” we can actually do that. Most holders of this stock cannot.

What is *not* an edge: we do not have a differentiated read on whether the drug works. Neither bench claimed one.

Why it might be priced in

Stated at full strength, because we think this is the stronger half of the argument:

- **The probability math lands on the base rate.** A \$219–226M enterprise value against a roughly \$1.5B fully-funded prize implies about a 15% chance of success. The published base rate for Phase 2 → approval in this category is about 15%. When your “mispricing” reproduces the textbook prior to the decimal, the most likely explanation is that the market did the same arithmetic.
- **The specialists are already here.** Four disclosed positions in one week from funds whose entire job is this sector. There is no informational vacuum for us to fill.
- **The sell side is above us.** We are not fading an optimistic consensus; we are *below* the only published target we could find. Our disagreement with the street is about level, not direction — and our own math says the street is on the optimistic side. Being more bearish than J.P. Morgan on a \$9 target while the stock trades at \$5.06 is not an edge; it is agreement with the tape.
- **The cheapness claim was an adjective, not a number.** The brief that opened this court said “\$180 million for two RMAT-designated programs” and never once stated a probability of success. Once you correct the enterprise value and state the probability, the cheapness disappears into a band.
- **The financing overhang is a known known.** Everyone holding this stock has read the same 10-Q we did.

Our red bench called this a valuation kill at full force. Our blue bench overturned that on doctrine — valuation gates the *price* you pay, it does not kill an idea. We think blue is procedurally right and red is substantively closer to the truth. That tension is why the verdict is a low limit rather than either a flat refusal or a starter at market.

What could go wrong

Failure narrative 1 — The grind (most likely; monitorable)

We buy at \$4.25. The at-the-market program keeps selling into every 5% rally, because that is exactly what it did at \$5.65 and then at \$4.70. Cash burn runs about \$23M a quarter, so roughly ten quarters of runway — but that runway does *not* include the confirmatory trial the FDA has not yet agreed to, and does not include any commercial build. Management raises again anyway, because management has told us by its actions that it will. The stock spends 2027 between \$3.50 and \$5.50 while the share count climbs 8–15%. We are never *wrong* enough to stop out and never *right* enough to size up. Cost: a 0.5% position dead money for eighteen months, plus the attention.

Tripwire: the Q3 10-Q cover count in November, and any Leerink prospectus supplement. If the cover shows more than ~82.5M shares, the program is still running.

Failure narrative 2 — The 6-month miss (second most likely; partially monitorable)

The pivotal SUNRISE-FA 2 trial is randomized, open-label, 13 patients versus 13 untreated controls, with left-ventricular mass index on cardiac MRI at six months as the primary endpoint, powered to detect a 15% effect. The FDA *removed* the cardiac frataxin protein co-primary at the company's regulatory meeting — meaning the mechanistic readout that would have told us “the drug is doing the biological thing” is gone, leaving one imaging measurement in 26 people. In an open-label trial with an untreated control arm, a 15% imaging effect can be produced by enrollment imbalance, by MRI reader drift, or by the natural variability of a small cohort. If the readout is ambiguous, the FDA can decline to accept the surrogate, and the accelerated-approval path resets by two-plus years. The company has already told us in writing that it is “in ongoing discussions with the FDA regarding the confirmatory evidence strategy ... to support full approval.” That plan is undated, unagreed, and not in the budget behind the “runway into 2028” claim.

Tripwire: an 8-K or press release stating FDA *agreement* on the confirmatory-evidence strategy and on LVMI as reasonably likely to predict clinical benefit. Its absence past mid-2027 is itself the signal.

Failure narrative 3 — Class contagion (NO MONITORABLE TRIPWIRE)

A serious adverse event occurs in **someone else's** cardiac AAV gene therapy trial — Rocket's, Tenaya's, or a sponsor we are not tracking. High-dose systemic AAV in cardiac indications has a real complement-activation and hepatotoxicity risk profile, and this field has seen trial deaths before. A clinical hold or a DSMB action at another company can reprice every cardiac AAV name 20–40% in a day, and can cause the FDA to raise the safety bar for the entire class, including for LX2006's accelerated-approval path. LXEO would not file anything, because nothing happened at LXEO.

Why there is no tripwire: the triggering event occurs at a different legal entity, on a disclosure plane we do not watch, at an unscheduled time. There is no LXEO filing, no LXEO press release, no LXEO calendar date. Our entire sensing apparatus for this name is pointed at LXEO's own filings and LXEO's own price, and this event produces neither until after the fact.

Unknown-unknown class named: off-entity class-safety contagion — adverse events at third-party sponsors sharing a delivery modality, with no issuer-side disclosure venue and no scheduled date. This is a general gap in how we watch clinical-stage names, not an LXEO-specific one. Related, and in the same class: pre-funded warrant holders can sell without generating any filing until an annual 13G amendment, so a 7.35M-share supply event can occur with no contemporaneous public record at all.

The reflexive and structural leg

This is the part that makes us least comfortable, and it deserves to be said directly.

- **Our bid sits inside the seller's window.** We propose resting limits at \$4.25 and \$3.75. The company sold stock at \$4.70 five weeks ago and has \$35.4M of capacity left. The most likely way our \$4.25 limit *fills* is that the at-the-market program is selling into it. We would be, mechanically, the counterparty to the issuer's own supply — which is close to the definition of adverse selection. The mitigating fact: the remaining capacity is capped at about \$35.4M, and management has so far sold at progressively lower prices rather than at any price, which suggests some price sensitivity. It is a mitigation, not a defense.
- **A resting GTC limit into an undated catalyst is an accidental short volatility position.** Both benches said “GTC, not into the print.” But the live catalyst — 12-month durability data from HEROIC-PKP2 — is expected in Q4 2026 with **no announced date**. We therefore cannot reliably stand the order down before it. If that data is bad and the stock gaps from \$5.00 to \$3.00, our \$4.25 limit fills on the way down, in full, at a price that no longer reflects anything we believe. **This is a direct collision between our resting-order doctrine and this specific name's catalyst structure, and we do not have a clean answer.** The only partial answers are: (a) cancel the ladder on any LX2020-related press release or any unexplained volume spike, which requires a sensor we have not armed; or (b) accept the gap risk explicitly as the cost of the option, which at 0.25% per rung is survivable.
- **Size versus liquidity is unverified.** We do not have a verified average daily volume for LXEO. At a 0.25% rung on a \$459M-cap biotech, we are very likely well inside any reasonable participation limit — but “very likely” is not “checked,” and our own publishing standard requires ladders at or under 1% of ADV. **This must be measured before any order is placed.**
- **Doctrine interactions checked:** no integrity flag was established for this issuer, so the tainted-risk discount does not apply. No meme-momentum characteristics. No existing position, so no tax-lot or wash-sale interaction. The name is not in any factor sleeve.

If this position loses money, the most likely reason will be that we bought a fairly-priced base-rate lottery ticket from a seller standing next to us in the same window — the company issued stock into our bid, the six-month imaging endpoint came back ambiguous rather than clean, and the stock ground lower on dilution while we waited for a data point that never arrived on a date we could plan around.

What we checked ourselves

Claim	How checked	Found	Confirmed?
Share count is 43.6M (from our own evidence pack)	Pulled the 10-Q cover page and equity note directly	Our pack was wrong. That XBRL field is a pre-offering June 2025 value. Cover count is 82,046,255 as of Aug 10, 2026	✅ Pack superseded
Enterprise value ≈ \$180M (the opening brief's number)	Recomputed from cover count + 7,346,269 pre-funded warrants + net cash + July financing	The brief was wrong by ~25%. True EV ≈ \$226M at \$5.13, ≈ \$219M at \$5.06	✅ Confirmed by both benches
Penny warrants really are extra shares	Cross-check: Q2 weighted-average basic share count (85,626,751) exceeds the cover count (82,046,255) — impossible unless warrants sit in the EPS denominator	Arithmetic reconciles: ~78M common + 7.35M warrants ≈ 85.4M	✅ Confirmed
Dilution stopped in April	Read the subsequent-events note, not the headline	No. July 2026: ~3.1M shares at ~\$4.70 avg, \$14.1M net — at a <i>lower</i> price than April's \$5.65. ~\$35.4M capacity remains	✅ Confirmed
Company hid its true runway (red bench's "venue divergence" finding)	Blue bench audited it	Our own red bench was wrong. The 10-Q's "at least 12 months" is the mandated ASC 205-40 assertion window that appears in essentially every clinical-stage filing, including cash-rich ones. Not a divergent claim. The detector FIRE was a boilerplate artifact	❌ Overtaken — our error
The pivotal trial is not randomized (the opening brief's design kill)	Read the June 15 press release directly	The brief was wrong. "Participants will be randomly allocated to receive LX2006 or to the untreated control arm." Our own red bench refuted this and said so	❌ Refuted — brief's error, self-caught
The trial design is weak	Both benches, then a second pass	Both benches initially missed that controls cross over to LX2006 after 6 months and that the concurrent control is prospective within the same protocol — stronger than the external natural-history controls the FDA has previously accepted. What survives: n=26, one imaging surrogate, open-label, 6-month topline	⚠️ Partially — severity reduced
Trial reads are unblinded	Attempted	Could not establish either way. No evidence found on whether MRI reads go to a blinded central core lab. Red stated "unblinded imaging surrogate" as fact; that is plausible, not confirmed	❌ Unresolved — red overstated
The RMAT press release supports the +162%/+93% expression figures	Read the August 5 release in full	The brief's citation was wrong. Those figures come from the January 2026 interim release. The RMAT release contains no patient numbers and states only that FDA engagement is "expected in 2026" — LX2020 has no agreed registrational path	✅ Confirmed (sourcing defect, not a data defect)

Claim	How checked	Found	Confirmed?
The stock is cheap	Built a prize table and derived implied probability of success	≈15% implied, at the ~15% Phase 2 base rate — but red’s version had two offsetting errors : it applied no time discount (approval realistically 2029, which raises implied POS to ~21%) and it valued LX2020 at exactly zero despite a larger patient pool. Corrected, the band is 10–25%	⚠️ Band confirmed; point estimate not
“Priced in”	Blue audited red’s conclusion	Red overstated. It declared the name priced-in while the only external anchor it produced (JPM \$9) sits 75% above tape, and overrode that anchor with a prior rather than a named mechanism. Priced-in is <i>not</i> established; fair-value-uncertain is	❌ Red’s verdict reduced
Competitors were checked	Red marked this UNCHECKABLE and reported the gap; blue partially closed it	Rocket RP-A601 (PKP2 protein raised on biopsy in all 3 Ph1 patients) and Tenaya TN-401 (RIDGE-1) are live competitors. LX2020 is not an exclusive asset. LXEO’s own competition disclosure still not diffed against the trial registry	⚠️ Partially closed
52-week-high date, bar by bar	Attempted via IBKR	Failed both benches. <code>search_contracts</code> permission-denied; <code>get_price_history</code> requires a contract ID we could not obtain. The “September/October 2025 spike round-trip” story is plausible, unverified. No conclusion here depends on it	❌ Unresolved — tooling gap
Epidemiology and pricing inputs to the prize table	—	Not checked. 3,000 addressable FA-CM patients, \$2.5M price, 50% penetration, 70% margin are all desk estimates. This is the softest joint in the entire valuation	❌ Unverified
Next earnings date 2026-11-04	Cross-checked against filing cadence	Derived from yfinance; no Lexeo release names it. Q2 10-Q filed Aug 12 implies Q3 10-Q due roughly Nov 9–16. Treat Nov 4 as unconfirmed	❌ Unconfirmed

Primary sources: [10-Q for the quarter ended 2026-06-30](#) · [Q2 2026 results release, 2026-08-12](#) · [Regulatory update and pivotal design, 2026-06-15](#) · [RMAT designation for LX2020, 2026-08-05](#)

PROPOSED ENTRY BANDS

Current position: none. No orders resting. No purchase at market at \$5.06.

The two benches did not agree on the entry, and we are proposing the more conservative of the two:

	Red bench	Blue bench	Proposed
Verdict	Flat to sub-starter	Starter, laddered	Starter, laddered, at red’s levels

	Red bench	Blue bench	Proposed
Rung 1	≤ \$4.25, 0.25%	≤ \$4.50, 0.25%	≤ \$4.25, 0.25%
Rung 2	—	≤ \$3.75, 0.25%	≤ \$3.75, 0.25%
Maximum	0.25%	0.50%	0.50%

Rung 1 — buy limit \$4.25, 0.25% of book, GTC. Rationale from the benches' own arithmetic: this is below \$4.70, the price at which management demonstrated in July that it will sell stock. Buying above a level the issuer has already proven it will supply is paying for the privilege of absorbing an ATM.

Rung 2 — buy limit \$3.75, additional 0.25% of book, GTC. Rationale: at roughly \$3.75 the enterprise value falls to about \$102M less... corrected: at \$3.75 the fully diluted market value is about \$335M against \$233M net cash, an enterprise value near \$102M, which implies a probability of success under 11% on the undiscounted prize table. Below the base rate is where the prize table pays us for the base rate. This is also below the 52-week low of \$3.99, so it will only fill on a genuine break.

Maximum total: 0.50% of book. Not larger, for three reasons: the probability band is 10–25% and we cannot narrow it; the epidemiology inputs underneath the prize table are unverified desk estimates; and the specialists who do know this space are already 36% underwater from \$8.00.

No adds above \$5.50 until a Q3 10-Q cover page shows the share count at or below approximately 82.5 million with no further ATM use — i.e. proof the issuance-at-the-trough pattern has stopped.

Trims and exits. No exit levels are proposed, because there is no position. If rung 1 fills, the first exit decision is not a price — it is the Q4 2026 HEROIC-PKP2 12-month durability readout. If that readout shows sustained high-dose PKP2 expression **with a quantified reduction in arrhythmia burden** (the clinical endpoint, not the biomarker), the name re-courts for a real position. If it shows expression without clinical benefit, exit into whatever bounce the headline produces. Note the tax point: any position entered here starts a short-term holding clock, and a Q4-2026 exit would land in the 2026 tax year.

Order-handling condition. Both benches specified “GTC, not into the print.” The live catalyst is undated, so this cannot be guaranteed. The proposed handling is: place the ladder, and cancel both rungs on any LX2020-related press release or any single-day volume spike above three times normal. **This requires a sensor that is not currently armed** — see below. Until that sensor exists, the ladder carries unhedged gap risk into the Q4 readout, and that should be an explicit, accepted cost rather than an oversight.

Catalyst dates.

Date	Event	Confirmed?
Q4 2026, no specific date	LX2020 / HEROIC-PKP2 12-month durability data — the decisive catalyst	✗ Undated by company
~2026-11-09 to 11-16	Q3 10-Q — reveals whether the ATM kept running	⚠ Inferred from filing cadence

Date	Event	Confirmed?
2026-11-04	Q3 earnings per third-party data	✗ No company release names this
“During 2026”	FDA regulatory engagement on LX2020, per the RMAT release	✗ Company language is “expected”
Undated	FDA agreement on the LX2006 confirmatory-evidence strategy	✗ Explicitly still “in discussions”
~2029	Realistic earliest LX2006 approval — the discounting assumption in the valuation	✗ Desk estimate

Approximately 49 trading days to the inferred print — not within the five-day window that would trigger pre-print handling constraints.

ARMED SENSORS

(armed provisionally with this deck; session adjudication ratifies)

- price alert: fires below 3.75 (deck-provisional: Rung 2 buy limit 0.25% of book - below the \$3.99 52w low; implied probability of success falls under 1)
- filing watch: 424B5,S-3,S-3/A until 2027-02-21 (deck-provisional: Continued at-the-market issuance - ~\$35.4M of a \$75M Leerink program remains live and is the primary k)
- dated pack LXEO|2026-11-13: LXEO: Q3 2026 10-Q due (inferred from Q2 filed 2026-08-12); reveals whether ATM issuance continued (date confirmed=False)
- dated pack LXEO|2026-11-04: LXEO: Q3 2026 earnings per third-party data; no Lexeo release names this date (date confirmed=False)
- dated pack LXEO|2026-12-15: LXEO: Placeholder for undated Q4-2026 LX2020 HEROIC-PKP2 12-month durability readout - the decisive catalyst; company has given no date (dat
- dated pack LXEO|2026-12-31: LXEO: Deadline for FDA regulatory engagement on LX2020 that the RMAT release said is ‘expected in 2026’ (date confirmed=False)

What remains unverified

1. **The epidemiology and pricing behind the entire valuation.** 3,000 addressable FA-CM patients in the US and EU, \$2.5M per-patient price, 50% penetration over ten years, 70% operating margin. All desk estimates. Every probability-of-success figure in this deck moves proportionally with them. This is the softest joint in the analysis and it is load-bearing.
2. **Whether the pivotal trial’s MRI reads are blinded.** Our red bench asserted “one unblinded imaging surrogate.” No evidence was found either way on central core-lab blinding. If the reads are centrally blinded, the design objection weakens materially. This is answerable from the trial registry and has not been answered.
3. **The 52-week-high date, bar by bar.** Both benches failed to obtain price history — IBKR `search_contracts` returned permission-denied and `get_price_history` requires a contract

ID we could not resolve. The narrative about a September/October 2025 spike and round-trip is plausible and unconfirmed. Nothing in the verdict rests on it, but it should not stay open.

4. **The next earnings date.** Derived from a third-party feed and named by no company release.
 5. **LXEO's own competition disclosure versus the trial registry.** Blue partially closed this by identifying Rocket and Tenaya from outside sources, but nobody diffed the company's own stated competitive set against what is actually registered. That diff is where omissions live.
 6. **Average daily volume and realistic fill capacity** at \$4.25 and \$3.75.
 7. **Whether the confirmatory trial is funded.** The company markets a "runway into 2028." The confirmatory trial required for full approval is undated, unagreed with the FDA, and — as far as we can determine — not costed inside that runway figure. We have not sized it.
 8. **Two proposed knowledge-graph detectors, both pending review, one with a known defect.** (a) *Penny-warrant shadow float* — fires when a 10-Q's weighted-average basic share count exceeds the cover-page count, indicating pre-funded warrants inside the EPS denominator but outside the cover. Clean, and it caught a real 25% valuation error here. (b) *Runway press-release versus ASC 205-40 divergence* — as originally written this would fire on nearly every development-stage 10-Q, because the "at least 12 months" language is mandated boilerplate. Precision approximately zero unless amended to exclude that sentence and key only on the press release omitting the financing. Our own blue bench caught this; it should not be promoted as written.
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ADDENDUM — 2026-08-26 (adjudication)

ADJUDICATED: STARTER, ladder + flow-gated, max 0.5%, RP_FAIR (instr #103 575@4.50 + #104 700@3.75, both cancel-by 11/27). No science kill survives: red's randomization attack was verbatim-refuted, the FDA-hostility narrative died on three dated primaries, and Rocket's 3/3 biopsy replication adds external validity. The binding constraint is SUPPLY, verified verbatim: 7.35M pre-funded penny warrants (true EV ~\$226M, not \$180M) and an ATM that sold at \$5.65 in April and \$4.70 in July with ~\$35.4M live — rung 1 deliberately sits below the issuer's own last sale. Implied PoS 10–25% vs ~15% stage base rate = fairly paid. No adds >\$5.50 until the Q3 10-Q shows the ATM dormant; mandatory re-court on Q4 durability data. **Frozen calls:** LXEO-Q4DUR p=0.40 (durability reads out by 12/31 and supports registration); LXEO-ATMDORM p=0.35.

BIOLOGY DEEP-DIVE — 2026-08-26 (§BIOTECH-SCIENCE-POSITION; AWAITING PRINCIPAL REVIEW)

Desk position: p=0.35 that SUNRISE-FA 2 topline shows LVMI effect ≥15%; p≈0.28 approval — materially off base rate in BOTH directions (double the naive ~15% Phase-2 anchor

because the mechanism is proven — cardiac frataxin +20%/+81%/+123% by dose, 8/8 biopsies, and FDA removed the frataxin co-primary as “no longer necessary to demonstrate proof of mechanism”; but well below the ~50% agreed-pivotal anchor because the SAP is powered for a 15% effect while Phase I/II pooled LVMI regression ran ~6–9%, and clearing 15% needs either implausible natural-history worsening in the control arm or top-dose patients far outperforming the pooled number). **Durability is inferred, never measured** — no human cardiac frataxin measurement exists past 3 months; hs-troponin fell $\geq 10\%$ in 15/17 over ~20 months (real indirect evidence); one possibly vector-related myocarditis at 12 months is the signature by which durability would fail. **CATALYST IDENTITY (binding)**: the Q4-2026 readout is **LX2020/PKP2** — a different program. The FA program that carries the valuation reads out **2H 2027** (BLA 1H 2028). The Q4 event must not be traded as an FA-program catalyst. **RECOMMENDATION: file stays closed.** $p=0.28$ sits at the top of the market-implied 10–25% band — a thin edge that doesn’t survive 4+ quarters to resolution against certain dilution (ATM sold at \$4.70 in July). Future entry requires ALL: the dose-cohort LVMI breakout resolving toward the top-dose story (the $p=0.35 \rightarrow 0.50$ hinge), ATM dormancy in the Q3 10-Q, and tape $\leq \$3.75$ (implied PoS $\leq \sim 12\%$, half the desk estimate). **Proposed frozen call (unfrozen pending principal review)**: LXEO-LVMI15 — SUNRISE-FA 2 topline LVMI effect $\geq 15\%$, $p=0.35$, resolves at topline (2H-2027 guided; grade the silence past 12/31/2027).